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The Rural Policy Research Institute Health Panel (Panel) was established in 1993 to provide science-based, objective policy analysis to federal lawmakers. The RUPRI Health Panel is pleased to respond to this Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13). Artificial Intelligence applications in clinical care and associated activities of healthcare organizations could contribute to reaching the Panel's vision of a High Performing Rural Health System, and a vision for the future of rural health. Our comments in this response draw on a report posted on the website of the National Advisory Committee on Rural Health and Human Services (NACRHHS); the RUPRI Center for Rural Health Policy Analysis contributed to the content of that report.
The NACRHHS identified policy priorities in rural health system development that would benefit from technologies, including use of AI (those that affect direct clinical care are highlighted):

- Improving transportation services in rural places, particularly for high-need patients
- Participating in new collaborations with other organizations (including community-based human service organizations), driven by data analytics
- Negotiating payment rates with commercial insurance plans, including Medicare Advantage plans and Medicaid Managed Care Organizations, and maximizing revenues through improved documentation (coding, other information related to patient chronic and episodic conditions)
- Using information technology to improve care management, particularly relevant in optimizing revenue in new payment models through improved coordination across all points of service delivery that contribute to sustained optimum health and well-being
- Adopting new care modalities in ways that enhance access to, and use of local services across the continuum that includes clinical care and human services
- Providing in-home health and human services
- Implementing virtual care, including 24-hour nursing care
- Improving navigation of multiple points of entry into the continuum of health and human services, making connections to the full array of potentially beneficial services regardless of the entry point

Overall themes of the report included the following:

- Quality access to broadband services is a major factor and potential barrier – up to 45 million American lacked availability of quality broadband services as March 2024 (FCC Press Release on March 2014)
- Implementing innovative technology requires leveraging existing alliances of hospitals

- Technology to improve efficiency of clinicians is a means of coping with workforce shortages (though far from sufficient) – ambient listening to populate health records being a specific example

General Panel Comments
Technology can be an equalizer but often first exacerbates disparities that exist. It can reinforce differences in access across geography. Specifically, technology that facilitates ease of access for well-insured populations may divert encounters away from local providers who serve all patients regardless of source of payment. That diversion could threaten financially viability of local providers. While this phenomenon could occur in any underserved area (including parts of urban areas), we call attention to potential unintended consequences in rural places. Intuitively, these challenges could be addressed by a combination of assuring access to the new technology for all residents regardless of source of payment and therefore precipitating an adjustment to use patterns that enhances access for all, or by increasing other sources of revenue for local providers to maintain their operations. We encourage further research and analysist to anticipate effects, monitoring implementation of technology to identify all consequences, and addressing unintended consequences through state and federal policies regarding through payment (Medicare and Medicare) and insurance regulation,

Q5. Supporting private sector activities to promote innovative and effective use of AI
To promote industry-driven testing we suggest allowing government agencies to work with private entities through cooperative research & development agreements, facilitating joint access to and use of government databases. We also suggest developing specific criteria focused on quality of care as AI products move from development to application. We specifically recommend that any such regulations be based on evidence of successful application in rural health care delivery. Analytical criteria should include affordability for healthcare organizations and patients, utility of the innovative technology to rural healthcare organizations (this may be function of scale and scope of services), and effects on long-term sustainability of access to local services. Further, success should be a combination of improved health outcomes and increased availability of services in rural communities.

Q6. AI tools with potential for greatest impact in rural settings.
As stated in reviewing themes from the NACRHHS, AI tools that enable clinicians to spend less time in administrative tasks and increased time in patient care would impact access to quality care in rural settings. Tools that facilitate communication between local primary care providers and other specialty clinicians would have an impact, to support local care and to coordinate care across the continuum through ease of patient transfer as necessary.

Q7. Administrative hurdles
Two hurdles are unique to small, independent rural healthcare organizations. First, access to high-speed broadband capacity remains a challenge, both for some clinics and for millions of patients in their homes. Second, the resources necessary to take full advantage of AI are lacking, both to support equipment and software, and personnel (data analysts) to take full advantage of new AI tools. Critically, administrative training and support will necessary to take full advantage of all new reimbursement codes associated with new technologies and to mirror systems used by payers such as claims review and pre-authorization.

Q9. Challenges within health care for rural patients and caregivers to address with AI
The following challenges could be addressed:

- Generating shorter wait times: AI tools that facilitate patient check-in and other pre-encounter administrative tasks could help reduce in-clinic wait times.
- More efficient and effective care: Efficiency and effectiveness could be improved with tools that enhance access to the right information at the right time during patient encounters.

Q9. Concerns related to the adoption and use of AI in rural settings
Rural healthcare patients often face long wait times, inefficient or ineffective care and high costs of care. AI can be leveraged and developed to enhance the effectiveness and efficiency of the healthcare delivered to rural patients which will shorten wait times and lower the cost of healthcare. However, the integration AI into the healthcare industry must be conducted through a careful and thought-out process. AI is a new and evolving technology. It can generate hallucinations or false results. Additionally, the use of improperly tested AI applications in the healthcare industry may lead to poorer health outcomes for patients. Further, enhanced use of AI in healthcare may lead to a lack of patient interactions with doctors, and accessibility concerns for patients who are less familiar with the technology. Finally, without aid from the government rural healthcare systems may not be able to benefit as much from the implementation of AI in healthcare when compared to urban areas due to a lack of infrastructure capacity.

Q 10. Research and Development
Several systematic reviews published by the NIH have concluded that the use of AI in clinical care settings can greatly enhance clinical decision-making processes and imaging/diagnosis processes (https://pmc.ncbi.nlm.nih.gov/articles/PMC10916499/). Further studies published by the NIH demonstrate that the use of AI in variety of clinical settings not only achieved comparable clinical outcomes to standard care but also created measurable improvements in QALYS and other effectiveness metrics. Additionally, these studies state that the use of AI leads to more efficient and effective care by enhancing clinical outcomes through improved diagnostic accuracy and personalized treatment while simultaneously reducing costs by optimizing resource allocation and streamlining care (https://pmc.ncbi.nlm.nih.gov/articles/PMC12381244/). However, there are costs to implementing AI. First there is a lack of published research discussing the upfront capital expenditures and integration costs required to implement AI in a clinical healthcare setting (https://pmc.ncbi.nlm.nih.gov/articles/PMC12381244/). Second, AI is an imperfect technology therefore, the use of AI may lead false or potentially missed diagnoses (https://www.nih.gov/news-events/news-releases/nih-findings-shed-light-risks-benefits-integrating-ai-into-medical-decision-making). Finally, the use of AI in the clinical setting will lead to social costs as some employee positions will be eliminated or greatly altered by the enhanced use of AI in the clinical setting.

Concluding comments.
The RUPRI Health Panel recommends that any innovation in AI be fully vetted with rural clinicians and patients before being endorsed for widespread use. We suggest a “rural proofing” that accounts for challenges of implementing new technologies in a resource-poor environments and that tests viability for the end users – clinicians and patients.

Respectfully submitted,

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